FOLFOX-4

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

FOLFOX-4 · active treatment cycle

Active medicines: Oxaliplatin + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before Day 1 chemotherapy Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	8 mg orally once	Before Day 1 chemotherapy Reconcile all steroid prescriptions; do not duplicate.
Dexamethasone	8 mg orally once daily	Days 2 and 3 after Day 1 chemotherapy Confirm the prescribed delayed prophylaxis and re-anchor it if treatment is delayed.
Metoclopramide	10 mg orally up to three times daily as required	Rescue only; duration to be completed by prescriber Review renal function, neurological contraindications and interactions.

Additional phase-specific support

- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.
- FOLFOX4 has Day 1 and Day 2 fluorouracil/folinic acid. Reconcile antiemetic cover across both days; do not issue a second independent Day 1 steroid course automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00210 v9b · NCCP NCIS antiemetics V8, Table 1; apply oral regimen guidance separately

Sources checked 2026-11-04 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 2 · FOLFOX4 fluorouracil/folinic acid

Active medicines: Folinic acid + Fluorouracil

Emetogenic risk: Low treatment-day risk; delayed Day 1 moderate-risk coverage still applies

Medicine	Dose / route	When
Dexamethasone	8 mg PO once	Day 2; already part of the Day 1 moderate-risk Days 2–3 plan Reconcile this single dose across both plans; do not issue an additional duplicate dose.
Metoclopramide	10 mg PO up to three times daily as required	Rescue only as prescribed Review persistent nausea, hydration and individual contraindications.

Additional phase-specific support

- FOLFOX4 Day 2 includes folinic acid and fluorouracil bolus followed by a second 22-hour infusion.
- No repeat oxaliplatin is scheduled on Day 2.
- NCIS multi-day guidance: cover the risk on each administered treatment day and delayed risk after treatment, using a reconciled prescription.
- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.

- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00210 v9b · p1 · NCCP NCIS V8 multi-day systemic anticancer therapy (SACT) principles

Sources checked 2026-11-04 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required